

Patient NAME : [REDACTED]  
DOB/Age/Gender : 55 Y/Female  
Patient ID / UHID : 16725937/OF16725937  
Referred BY : Dr. SATENDRA SINGH ATTRY ..  
Sample Collected : May 11, 2026, 09:04 AM

Report STATUS : Final Report  
Barcode NO : 57948112  
Sample Type : Whole blood EDTA  
Report Date : May 11, 2026, 02:24 PM.



| Test Description | Value(s) | Unit(s) | Reference Range |
|------------------|----------|---------|-----------------|
|------------------|----------|---------|-----------------|

**Hemoglobin (HB)**

|                                         |        |      |             |
|-----------------------------------------|--------|------|-------------|
| Hemoglobin<br>Cyanide free colorimetric | 5.1 L* | g/dL | 12.0 - 15.0 |
|-----------------------------------------|--------|------|-------------|

RESULT RECHECKED

KINDLY CORRELATE CLINICALLY.

**Interpretation:**

- 1.Hemoglobin is a protein contained in red blood cells which is responsible for oxygen delivery to tissues.
- 2.To achieve adequate tissue oxygenation , a sufficient level is required.
- 3.Low levels indicate anemia and further testing may be required to evaluate the cause.
- 4.High levels may be seen in persons living at high altitude , dehydration, smoking and some blood disorders.

Note :- (H\* - High , L\* - Low ,CL\* - Critical Low,CH\* - Critical High)



**Dr. Nuzra Fazal**  
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**Consultant Pathologist**



Booking Centre :- DEE JEE REDCLIFFE COLLECTION CENTRE , F1267, Center name - DEE JEE LAB , F1267  
Processing Lab :- Redcliffe Lifetech Pvt Ltd, 43, Shiv Puri, Meerut, Uttar Pradesh- 250001

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Sample Type : Whole blood EDTA  
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|------------------|----------|---------|-----------------|

**Erythrocyte Sedimentation Rate (ESR)**

|                                                             |       |       |        |
|-------------------------------------------------------------|-------|-------|--------|
| ESR - Erythrocyte Sedimentation Rate<br>MODIFIED WESTERGREN | 78 H* | mm/hr | 0 - 19 |
|-------------------------------------------------------------|-------|-------|--------|

**Interpretation:**

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

**Reference-** Dacie and Lewis practical hematology

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Barcode NO : 57948111  
Sample Type : Serum  
Report Date : May 11, 2026, 02:13 PM.



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**Uric Acid**

|                             |     |       |           |
|-----------------------------|-----|-------|-----------|
| Uric Acid<br><i>Uricase</i> | 4.4 | mg/dL | 3.5 - 7.2 |
|-----------------------------|-----|-------|-----------|

**Interpretation:**

Serum uric acid levels are very labile and show day to day and seasonal variation in some people. Levels are also increased by emotional stress, total fasting and increased body weight. Serum uric acid levels are used to diagnose and monitor treatment of gout, monitor chemotherapeutic treatment of neoplasms to avoid renal urate deposition with possible renal failure.



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**C-Reactive Protein (CRP), Quantitative**

|                                          |         |      |     |
|------------------------------------------|---------|------|-----|
| CRP (Quantitative)<br>Immunoturbidimetry | 15.4 H* | mg/L | <10 |
|------------------------------------------|---------|------|-----|

**Interpretation:****Increased CRP level:**

1. A high or increasing amount of CRP in the blood suggests the presence of inflammation but will not identify its location or the cause.
2. Suspected bacterial infection—a high CRP level can provide indication that patient has an infection.
3. Chronic inflammatory disease—high levels of CRP suggest a flare-up if you have a chronic inflammatory disease or that treatment has not been effective.

If the CRP level is initially elevated and drops, it means that the inflammation or infection is subsiding and/or responding to treatment.

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**Rheumatoid Factor (RF), Quantitative**

|                                                              |     |       |        |
|--------------------------------------------------------------|-----|-------|--------|
| RHEUMATOID FACTOR, Quantitative<br><i>Immunoturbidimetry</i> | 0.1 | IU/mL | 0 - 16 |
|--------------------------------------------------------------|-----|-------|--------|

**Interpretation:**

Approximately 85% of patients with Rheumatoid arthritis have detectable RA. It may also be seen in other medical conditions like Sjogren's syndrome and SLE.



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**Thyroid Profile Total**

|                                                      |                |       |             |
|------------------------------------------------------|----------------|-------|-------------|
| Triiodothyronine (T3)<br>CMIA                        | 110.8          | ng/dL | 35 - 193    |
| Total Thyroxine (T4)<br>CMIA                         | 6.8            | µg/dL | 4.87 - 11.2 |
| Thyroid Stimulating Hormone (Ultrasensitive)<br>CMIA | <b>10.7 H*</b> | mIU/L | 0.35 - 4.94 |

**Interpretation:**

| Pregnancy     | Reference Range TSH |
|---------------|---------------------|
| 1st Trimester | 0.1 - 2.5           |
| 2nd Trimester | 0.2 - 3.0           |
| 3rd Trimester | 0.3 - 3.0           |

**Note:** TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

**Clinical Use:**

1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

**Increased TSH:** Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance

**Decreased TSH:** Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. **Primary Hypothyroidism:** High TSH levels.
2. **Secondary/Tertiary Hypothyroidism:** Low TSH levels.
3. **Euthyroid Sick Syndrome:** Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

| TSH    | T4             | T3             | Interpretation                                                                                                      |
|--------|----------------|----------------|---------------------------------------------------------------------------------------------------------------------|
| High   | Normal         | Normal         | Mild (subclinical) hypothyroidism                                                                                   |
| High   | Low            | Low Or Normal  | Hypothyroidism                                                                                                      |
| Low    | Normal         | Normal         | Mild (subclinical) hyperthyroidism                                                                                  |
| Low    | High Or Normal | High Or Normal | Hyperthyroidism                                                                                                     |
| Low    | Low Or Normal  | Low Or Normal  | Nonthyroidal illness; pituitary (secondary) hypothyroidism                                                          |
| Normal | High           | High           | Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function) |

**References**

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568

\*\*\* End Of Report \*\*\*

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and rate overall experience.

How likely are you to recommend Redcliffe Labs to your family & friends



Not Likely

 Very Likely



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